



Communication and Documentation Mental Health Hospital in the Home (MHHiTH) (Mental Health) Procedure

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

Mission of Service

To offer a contemporary, recovery-oriented alternative to inpatient admission for RKPG mental health patients by providing safe, high quality, evidence based and responsive mental health hospital level care in their home or usual place of residence.

Aim

- Provide instructions on how the MHHiTH team communicate within a multidisciplinary setting.
- Staff will follow the [DoH Mental Health bed Access, Capacity and Escalation State-wide Policy](#) and use the standardised clinical documentation forms for adult mental health [Department of Health \(DoH\) State-wide Standardised Clinical Documentation \(SSCD\)](#).

Risk

Non-adherence to this guideline may result in miscommunication, compromised patient care and staff safety.

Communication

Clinical handover/review

- All clinical handovers will be undertaken using iSoBAR (Identification, Situation, Observations, Background, Assessment, Recommendations) in accordance with the following policy documents:
 - * [RKPG Clinical Handover-Nursing and Midwifery](#)
 - * [RKPG Medical Clinical Handover](#)
- Clinical handover of care will involve documentation on iSOFT.

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Apr 2021	Revisions	NA	National Standards 1, 5, 6
This version	May 2024	Revision Due	May 2027	
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- The clinicians will handover in and review patient sin the following order:
 - * Triage of patient's requiring most urgent review.
 - * Identification of patient.
 - * Situation/presenting problem for the patient.
 - * Patient background/history.
 - * Observations.
 - * Assessment risk:
 - Patient of concern status.
 - Crisis plan.
- Recommendations - plan/outcome for patient care.
- Expected date of discharge (EDD).
- All other accompanying documentation must be is complete, psychiatric services online information system (PSOLIS) updated.

Clinical handover

- MHHiTH nursing clinical handover will be performed at times suitable to the team.
- Handover will be performed via phone, face to face, video conference.
- Handover will be documented iSOFT and the running sheet and updated as required by allocated nurse/medical team or senior nurse in charge of shift.
- MHHiTH Shift Coordinator will hand over patient of concern handover via phone to the duty medical officer (MO)/Registrar for Mental Health and Mimidi Park Shift Coordinator.
- 24 hour collated report:
 - * MHHiTH staff are required to complete bed status report daily which outlines bed occupancy including any admissions, discharges, or patients on leave. See Appendix A.
 - * The bed report is saved on the shared W-Drive folder for access afterhours.
 - * The bed report will be sent out via email at the end of shift by the senior nurse to a restricted group of senior clinicians including Program Manager, Mental Health Medical Co-Director, Duty Nurse Manager (DNM) at the end of each day Mimidi Park Shift Coordinator and DMO.

Medical handover

- In hours medical handover will occur via face to face, phone or videoconference (see below for guideline) at times suitable to the medical team.
- Afterhours and weekend handover will be given to the DMO/Registrar for Mental Health. They will update the Consultant on call, as required, with patients of concern.
- Handover will be performed in an iSoBAR format as per [RKPG Medical Clinical Handover](#).

Multidisciplinary team (MDT) review

- MDT review of client assessment is undertaken to ensure the most appropriate clinicians are involved in the care of the client. These will occur as per [RKPG Intake Meeting, Case Allocation and Case Review Guideline](#).
- MDT will occur biweekly and be scheduled to suit team.

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- They will comprise of patient review and care planning, including discharge planning for inpatients and waitlisted patients (those waiting to come into the service).
- Each patient will have a minimum of one discussion at MDT per week.

Communication/documentation and mobile technology

MHHiTH will have access to mobile technology including laptops and data enabled mobiles. The MHHiTH team will use this technology to facilitate their communication and documentation processes. This will allow for more timely communication and documentation as well as further encourage collaborative care planning.

Videoconferencing guidelines

- Use of telehealth must be as per:
- [RKPG Telehealth: Community Mental health \(Mental Health\) Guideline](#)
- [South Metropolitan Health Service \(SMHS\) In Home Video Consultation Guidelines](#)
- **Note:** Clinicians need to formulate a plan at the beginning of the teleconference in case technology fails. For example - request a contact number from patient and tell them you will call them if you are disconnected.
 - * **Clinician to patient/carer:**
Clinician can set up appoint via link to patient/carer, patient/carer will click onto link and connect to videoconference with clinician.
 - * **Clinician to clinician with patient carer present:**
Video conferencing for clinician to clinician with patient carer present can be done using the approved platform for South Metro Health Service (SMHS).
 - * **Clinician to clinician:**
Clinician to Clinician video conferencing can be done using approved platform as per SMHS recommendations.

Visitation/Telehealth

- Use of telehealth is only to be used as an adjunct, not as a substitute for visitation.
- All visits must be conducted face to face at the patient's designated location to be determined as MHHiTH visitation and be recorded MHHiTH day as per [DoH Patient Activity Data Policy Information Compendium 2023](#).

Contact numbers patient and carer

- Patient and carers are to call the MHHiTH number provided on admission to MHHiTH.
- Evening and weekend/public holidays the main contact for MHHiTH will be the senior nurse on duty for MHHiTH on the same number.
- Afterhours the senior nurse will be responsible for diversion of the CNM mobile phone to Mimidi Park Shift Coordinator's dect phone.

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Documentation

Referral

- The Mental Health Assessment (SMHMR902) a detailed discharge summary or detailed medical review, Risk Assessment and Management Plan (RAMP) and MHHiTH referral form which will be emailed to park.mhshithtrriage@health.wa.gov.au where the referral will be assessed and triaged for admission as per clinical need.
- The patient will be put on Enterprise Bed Management (EBM) for the CNM or Mental Health Clinical Nurse Manager Patient Flow (MHCNMPF) to allocate a bed. See [RKPG Referral, Admission and Discharge: Mental Health Hospital in the Home \(Mental Health\) Policy and Procedure](#)

Admission

- Upon admission to the MHHiTH service the team will be expected to commence discharge planning.
- Patient consent form will need to be completed RGMR54.
- If **patient is transferring from inpatient unit** there is an expectation the transferring team have commenced discharge planning including discharge summary on the Notification and Clinical Summary (NACs).
- The MHHiTH allocated nurse will complete a MHHiTH care plan package at the time of referral to MHHiTH, ensuring the following documentation is included:
 - * MHHiTH nursing interventions.
 - * Patient contact numbers.
 - * Directions to patient's residence and complete the home visiting safety assessment (RGMR119), this will be kept at the front of the patients file, followed by PSOLIS alerts for the time of patient admission to the service [RKPG Community Home Visiting](#)

Nursing admission

The following must be completed:

- Initial nursing assessment will occur within 24 hours of patient admission to the MHHiTH service.
- Patient identification see [RKPG Patient Identification](#)
- Adult mental health assessment (SMHMR902) which includes carer/family/NOK details, mental state examination, legal status.
- Risk Assessment (SMHMR905).
- Mental Health Risk Assessment and Management Plan (SMHMR905).
- Drug and Alcohol screening (Using DAST if indicated).
- Mental Health Physical Appearance Form (SMHMR904).
- Establishment of a biopsychosocial management plan.
- Observations attended and documented in patient notes.
- Mental health Care Transfer Summary (SMHMR916).
- Medication History and management plan (RGMR169.1).
- Treatment Support and Discharge Plan.
- PSOLIS Client Management Plan.
- PSOLIS Crisis Plan (if appropriate).

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- Activation PSOLIS.
- National outcome case-mix collection (NOCC) measures.
- Consent to release information (RGMR30.F).
- Safety assessment to be documented in progress notes and recorded in iSOFT.
- iSoBAR handover to be completed iSOFT.
- Enter the consumer's details into PSOLIS and complete other admission and documentation requirements as per the Mental Health Inpatient Admission Documentation including, the [DoH SSCD](#).
- Expected date of discharge (EDD) will be entered into EBM.

Medical admission

- The first medical review will be conducted within 48 hours of patient being admitted into the service. This will entail:
 - * Medication review (at first medical appointment) and prescriptions as required.
 - * Mental health Physical Examination Form (SMHMR903) (following completion of the initial assessment process the MHHiTH service will document a summary of presenting problems and relevant history).
 - * Completion of plan of care with interventions as decided with input from treating nurse.
- Pharmacy will complete medication reconciliation within 48 hours of admission.
- For Mental Health forms see [DoH SSCD](#).

Documenting provision of care

- Identified patient risks and other risks associated with provision of care must be clearly documented in patient's medical record and communicated to the treating team.
- Every visit, telephone call, videoconference, medical review, consultation, referral and follow up care must be documented in the Inpatient Progress (RGMR 55A), section of the MHHiTH notes and updated on PSOLIS.
- iSoBAR handover will be documented on iSOFT.
- Patient file will remain on site with only copies of medication chart and essential documentation taken with clinician to home visit.
- All other required documentation will be accessed and updated digitally using laptop at patient's home or on site (if hard copy required) at MHHiTH office.

Discharge

- The decision to discharge the patient from MHHiTH will be made by the treating team(s) and documented by a MO.

Final MHHiTH visit

- The MHHiTH allocated nurse will document in the patient file the provision of any last treatment and if any additional information was provided to the patient/carer regarding follow up care. Complete NACs summary.
- Medication list current up to date.

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- A discharge summary will be completed by the medical team; however, members of the multidisciplinary team (i.e. Pharmacy, Case Manager) will be required to make contribution to the document. Refer to:
 - * [SMHS Discharge Policy](#).
 - * [RKPG Patient Discharge Policy & Procedure](#).
- A copy of the discharge summary will be available within 24 hours of discharge with a copy given to the patient, their CMHS and General Practitioner (GP) as per:
 - * [SMHS Discharge Policy](#).
 - * [RKPG Patient Discharge Policy & Procedure](#).
- Following discharge from MHHiTH:
 - * The patient's documentation will be filed in the patient's medical record and updated on PSOLIS as per [RKPG Documentation Clinical Practice Standard \(CPS\)](#).
 - * The administration person responsible for completing Mimidi Park PaRK discharges afterhours will discharge from webPAS in real time, PSOLIS may be done retrospectively.
- Documentation must comply with [RKPG Documentation CPS](#).
- Confidentiality and privacy of patient information will be maintained according to the [DoH Patient Confidentiality Factsheet](#).

Transfer of care

- Care Transfer Summary (SMHMR916) completed.
- The MHHiTH team will liaise with relevant discharge services to ensure a seamless transfer of care.
- Where the patient is being discharged back to the care of a Community Mental Health Service, a discharge handover meeting should be arranged (where possible), in the days leading up to discharge to facilitate continuity of care.

Medical records

- The patient's medical records will remain in the MHHiTH office for the duration of the MHHiTH episode of care.
 - * Access to medical records will be preserved for Health Information Management Service (HIMS) for outpatient appointments.
 - * If the patient requires readmission to RKPG Mimidi Park PaRK (within 72 hours of discharge), the medical record must be retrieved from the MHHiTH office (day or night).
- After the patient has been discharged from the MHHiTH Program, the medical record will be returned to the patient's medical team for the discharge summary to be completed according to [RKPG Patient Discharge](#).
- Confidentiality of the medical record and associated clinical file notes will be maintained. Clinicians must document in the patient's medical records after every visit and update PSOLIS and as per [RKPG Documentation CPS](#)

Compliance monitoring

Compliance with this procedure will be monitored by any clinical incidents via Datix.

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References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. (Version 2) 2021. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>

Links

[DoH Acceptable Use of Information and Communications Technology Policy](#)
[DOH Chief Psychiatrist's Standards for Clinical Care](#)
[DoH Information and Communications Technology Governance Policy](#)
[DoH Information Security Policy](#)
[Department of Health \(DoH\) State-wide Standardised Clinical Documentation \(SSCD\) for Mental Health Services](#)
[DoH Patient Confidentiality Factsheet](#)
[DoH Patient Activity Data Policy Information Compendium 2023](#)
[DoH State-wide Standardised Clinical Documentation](#)
[DoH Mental Health bed Access, Capacity and Escalation State-wide Policy](#)
[RKPG Clinical Handover-Nursing and Midwifery](#)
[RKPG Community Home Visiting Policy](#)
[RKPG Documentation Clinical Practice Standard \(CPS\)](#)
[RKPG Evening and After Hours MH HITH](#)
[RKPG Intake Meeting, Case Allocation and Case Review Guideline](#)
[RKPG Medical Clinical Handover](#)
[RKPG Medication Administration Clinical Practice Standard](#)
[RKPG Patient Discharge](#)
[RKPG Patient Identification](#)
[RKPG Physical Health Care: Community Mental Health Consumers \(Mental Health\) Policy and Procedure](#)
[RKPG Referral, Admission and Discharge: Mental Health Hospital in the Home \(Mental Health\) Policy and Procedure](#)
[RKPG Telehealth: Community Mental Health \(Mental Health\) Guideline](#)
[SMHS Community/home visiting Policy](#)
[SMHS Discharge](#)
[SMHS In Home Video Consultation Guidelines](#)
[SMHS Telehealth Strategic Planning](#)
[SMHS Telehealth Tips](#)

National Standards



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Appendix A: Running sheet

RkPG Mental Health Services: MHHiTH Running Sheet					Date:	Shift:
Staff Name:		Designation:				
Inpatients :						
Name & AGE	URMN Suburb	Presenting problem/ History	Plan/Outcome	Expected Discharge Date (EDD)		
		Admission date:	Risk:			
		Admission date:	Risk:			
		Admission date:	Risk:			
		Admission date:	Risk:			
		Admission date:	Risk:			
		Admission date:	Risk:			
		Admission date:	Risk:			
		Admission date:	Risk:			
		Admission date:	Risk:			
Discharges:						
Name & Age	URM Suburb					
Wait listed Patients Prioritised for the service :						
Name & AGE	URM & Suburb	Presenting Problem:	Current Location			
Template available on W-Drive in GDD file under "MHHiTH Running Sheet". Please save this in W-Drive under "MHHiTH Running Sheet "						

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